



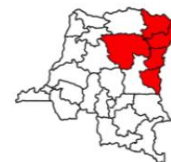
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°117/MVEBDB/08/09/2026

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HIGHLIGHTS (last 24 hours)

Eighty-six (86) new confirmed cases of Ebola Virus Disease (EVD), including 19 community deaths, were recorded as of September 8, 2026, in the provinces of Ituri (55 cases), North Kivu (27 cases), Haut-Kivu (27 cases), and South Kivu (27 cases). Uélé (2) and Tshopo (2). On the other hand, 24 deaths were recorded within the CTE. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6,843	ACCUMULATIVE DEATH CONFIRMED 3,310	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 833	ACCUMULATIVE HEALED 1,611	TRACKING RATES CONTACTS (Of the Day) 86.5%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



61 out of 151 health zones affected

(40.4%)



Summary of key points from the last 24 hours

For the day of September 8, 2026, **86 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including 19 **community deaths**.

These cases come mainly from the provinces of **Ituri (55 cases)** and **North Kivu (27)**, **Haut Uélé (2)** and **Tshopo (2)**. On the other hand, **24 intra-CTE deaths** were recorded, including 21 in Ituri and 3 in North Kivu, bringing the **daily total of deaths to 43**.

Meanwhile, **21 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (11 in Haut Uélé, 9 in Ituri, and 1 in North Kivu).

As of September 8, 2026, the DRC has accumulated **6,843 confirmed cases, including 3,310 deaths**, representing a case fatality rate of **48.4 %**. The Ituri province has remained the epicenter since the beginning of the epidemic and alone accounts for **79.8%** of confirmed cases reported across the six affected provinces. Furthermore, it holds **64.0%** of the new cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, **61 out of 151 health zones (40.4%)** remain affected in the 6 affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 8, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	55	5461	2,481	45.4%	28/36 (77.8%)
North Kivu	27	1093	709	64.9%	16/34 (47.0%)
Haut-Uélé	2	258	107	41.5%	6/13 (46.1%)
Tshopo	2	24	9	37.5%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	86	6,843	3,310	48.4%	61/151 (40.4%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 8, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5461	2481	45.4%	55	10	21	31
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	176	39	22.2%				0
Boga	2	2	100.0%				0
Bunia	1523	476	31.3%	21	5		5
Damascus	32	9	28.1%	2	0		0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	118	80	67.8%	1	1		1
Lita	247	122	49.4%	2	0		0
Logo	13	5	38.5%				0
Lolwa	25	7	28.0%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	44	24	54.5%	2	1		1
Mangala	297	141	47.5%	2	0		0
Mongbwalu	640	293	45.8%	2	2		2
Nia-Nia	228	115	50.4%	1	0		0
Nizi	723	273	37.8%	14	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0
Rwampara	999	348	34.8%	7	1		1

Tchomia	63	25	39.7%	1	0		0
To ventilate	N / A	408	N / A	0	0	21	21
North Kivu	1093	709	64.9%	27	9	3	12
Blessed	185	138	74.6%	4	1	2	3
Good	9	4	44.4%				0
Butembo	217	148	68.2%	6	2		2
Goma	1	0	0.0%				0
Kalunguta	20	9	45.0%	2	0		0
Katwa	477	305	63.9%	9	3		3
Kayna	1	0	0.0%				
Kyondo	25	17	68.0%	2	1	1	2
Lubero	7	4	57.1%	1	0		0
Mabalako	10	7	70.0%				0
Manguredjipa	4	3	75.0%				0
Masereka	15	6	40.0%				0
Musienene	101	55	54.5%	1	1		1
Mutwanga	2	2	100.0%				0
Oicha	9	6	66.7%				0
Vuhovi	10	5	50.0%	2	1		1
Haut-Uélé	258	107	41.5%	2	0	0	0
Boma Mangbetu	29	14	48.3%				0
Gombari	3	1	33.3%				0
Isiro	92	37	40.2%				0
Pawa	46	23	50.0%				0
Rungu	2	1	50.0%				0
Wamba	86	31	36.0%	2	0		0
Tshopo	24	9	37.5%	2	0	0	0
Bafwasende	4	0	0.0%	2	0		0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6843	3310	48.4%	86	19	24	43

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 8, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	20	5	25	19	5	0	0	24	16
Bas Uélé	39	0	39	5	0	33	0	5	1
Ituri	898	66	964	152	45	12	21	197	131
North Kivu	961	45	1006	132	45	814	0	177	78
South Kivu	6	0	6	0	0	6	0	0	0
Tshopo	99	1	100	4	1	95	0	5	3
Total General	2023	117	2,140	312	96	960	21	408	229

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

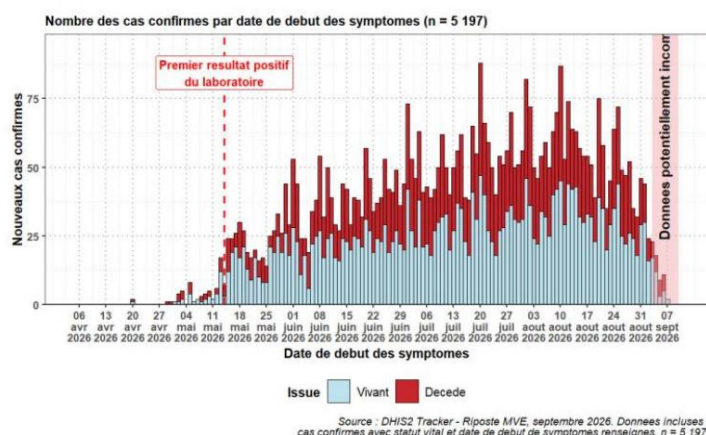
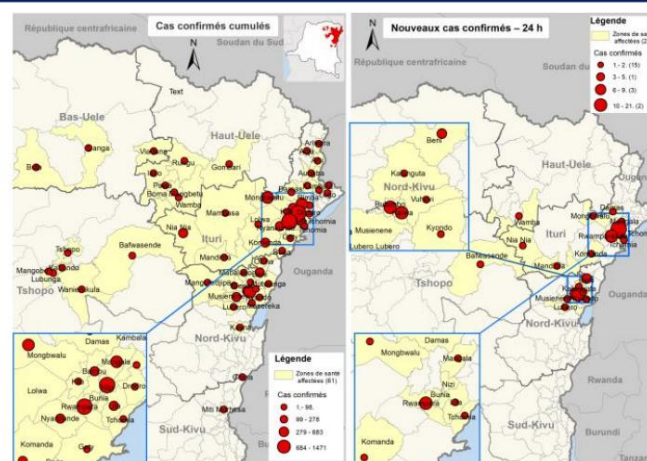


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.
- Unloading ceremony for 11 healed individuals by the provincial coordination team Upper Uélé.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

As of September 8, 2026, 2,140 alerts were reported in 6 provinces, 1,389 (64.9%) could be verified. All 408 alerts validated as suspected cases were investigated (100%) and 229 of them were transferred to CT/CTE (Table 3).

Of the 25,108 contacts to be followed up for the day of September 8, 21,723 of them were seen, representing a daily proportion of **86.5%**, above the threshold of 85%; **90.1% in Ituri (11,210/12,446)**, 84.3% in North Kivu (8,733/10,358), 71.1% in Bas Uélé (108/152), 83.2% in Tshopo (223/268) and 76.9% in Haut Uélé (1,449/1,884).

1.1.3. Challenges

Insufficient teams dedicated to contact tracing activities make contact tracing ineffective, particularly in Bas-Uélé, despite improvement efforts.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

As of September 8, 2026, seven alerts were notified to the various PoE/PoCs, five in Bas-Uélé and two in Ituri. **In Ituri**, a live alert was validated and transferred to the Kasenyi CTE (originating from Tchomia, Tchomia PoC), and a body was intercepted at the Tchai PoC (Makabo Health Area, Rwampara Health Zone), en route from Gety to Bunia, for which a Discharge Summary Investigation (DSI) is underway. **In Bas-Uélé**, the five alerts (three at the PK12 Isiro PoC in the Viadana Health Zone, one at the PK3 Mboti PoC in the Ango Health Zone, and one at the PK2 Mokeni PoC in the Titule Health Zone) were all invalidated after verification.

Table 4. Status of key indicators for PoC/PoE by province, as of September 8, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	79.5% (58/73)	100% (19/19)	55.5% (5/9)	88.8% (16/18)	64.0% (7/11)	ND	84.6% (99/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	ND	100%
Number of people who have switched to PoE/PoC	124 889 88 561		7,129	14,998	12,208	6034	253,819
% of travelers screened	97.00% 97.90%	98.70%	99.30%	99.00%	98.00%		97.60%
% of travelers who washed their hands	97.60% 97.90%	98.70%	99.50%	99.00%		ND	97.90%
% of travelers made aware: 99.40%	97.90%	100.00%	100.00%	99.00%		ND	98.90%
Number of alerts notified	2	0	0	0	0	5	7
Number of live alerts validated (suspected cases)	1	0	0	0	0	0	1
Number of bodies recovered today	1	0	0	0	0	0	1
% of suspected cases transferred to CT/CTE	100%	N / A	N / A	N / A	N / A	N / A	100%
% of lifeless bodies swabbed/secured	100%	N / A	N / A	N / A	N / A	N / A	100%
% of suspected cases investigated within 24 hours	100% NA		N / A	N / A	N / A	100%	100%

1.2.2. Challenges

Operationalization of PoE/PoC remains suboptimal: low reporting completeness in South Kivu (55.5%, 5/9) and Haut-Uélé (64.0%, 7/11); insecurity continues to hinder the commissioning of the Avakubi PoC (Ituri); in Tshopo, only 18 of the 30 planned PoCs remain activated (completeness 88.8%, 16/18).

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 55 new positive results** (45 living and 10 deaths) out of 338 new samples received and analyzed (**positivity: 16.3%**).

- **North Kivu : 27 new positive results** (18 living and 9 dead) out of 137 samples analyzed (**positivity of 19.7%**).
- **Haut-Uélé: 2 new positive results** (2 live) out of 3 samples analyzed (**positivity: 66.7%**).
- **Bas-Uélé: 2 samples** received and tested (2 live), **all came back negative**.
- **Tshopo: 2 new positive results** (2 live) out of **27 samples analyzed (positivity of 7.4%)**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 15 rings were opened out of the 59 expected (25.4%), 58 households decontaminated out of 73 identified (79.5%) and 10 FOSA decontaminated out of 10 (100%); 60 IPC kits were provided to households in the Mongbwalu15 health zone. As for EDS, 75 alerts were recorded, 62 EDS carried out (82.7%) and 51 bodies swabbed out of 62 (82.3%).
- **In North Kivu**, 27 of the 30 planned infection control rings were opened (90%), and 13 of the 47 identified households were decontaminated (27.7%). Training for IPC/WASH teams in Butembo and Katwa continued (Day 2). Training for 30 frontline healthcare providers in Kalunguta and 60 hygiene workers in Kyondo was launched. The EDS (Emergency Health Data) pillar received 42 alerts (23 were postponed from the previous day), conducted 44 EDS, and swabs all 42 bodies (100%).
- **In Tshopo**, decontamination of 2 premises at CM Shukrani (AS of Umoja, ZS of Kabondo); monitoring and support of 12 ESS in 5 health zones (Makiso-Kisangani, Mangobo, Tshopo, Kabondo and Lubunga) with the briefing of 128 providers on IPC measures; 8 triage units installed out of 12 expected and 500 litres of water supplied to Wanie-Rukula; 1 ring opened in Kabondo.
- **In Haut-Uélé**, 3 rings expected, all were opened (100%); 5 households decontaminated out of 6 identified (83.3%) and 2 out of 2 FOSA (100%), without provision of kits to households; 5 EDS were planned, 3 were carried out, 2 failures following community resistance in the ZS of Isiro.
- **In Bas-Uélé**, briefing of 48 healthcare providers in 8 health establishments in 4 health zones and of 67 detainees of the Buta central prison, with provision of hand washing devices and IPC supplies; bio-cleaning of the premises of the Ganga-Dingila BCZS.

1.4.2. Challenges

- **In Ituri** : Low opening of the rings (25.4%, 15/59) and insufficient decontamination teams in Fataki, Mangala and Komanda; Lack of mobility in Bambu, Mandima; Non-completion of activities following the break in inputs in Mangala and Mandima.
- **In North Kivu** : insufficient decontamination of identified households (13/47) and teams still need to be activated and trained in Biena, Masereka, Kyondo, Mabalako and Vuhovi.
- **In Tshopo** : insufficient IPC inputs in social and solidarity economy (SSE) sectors, untrained service providers and The incinerator at HGR Makiso-Kisangani is out of order.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 87 new admissions to the various dedicated facilities compared to 89 discharges. of which 9 have recovered. Thus, 471 patients are hospitalized out of 983 beds, representing an occupancy rate of 47.9%; 417/448 (93%) patients received hot meals; 14/14 (100%) children are receiving LNPE/SLM at the CTE, 3/3 (100%) at the nursery, and 27/27 (100%) in the community; 7

transfers were organised (1 from CT Mangala to CTE Nizi and 6 from CT Kigonze to CTE HGR Bunia).

- **In North Kivu**, 88 new admissions and 55 discharges were recorded, including 1 recovery; 287 patients are hospitalized, resulting in an overall occupancy rate of oversaturation (130.5%; 287/220 beds available).
- **In Haut Uélé**, there were 3 new admissions compared to 16 discharges (11 recovered). Thus, 49 patients are hospitalized out of 134 beds, representing an occupancy rate of 36.6%.
- **In Tshopo**, 3 new admissions were recorded and no discharges (no recoveries). Seven (7) patients are in isolation for 25 beds, representing an occupancy rate of 28.0%.
- **In South Kivu**, no new admissions during the day, one discharge and 19 patients are in isolation (Occupancy rate: 76.0%).

1.5.2. Challenges

Treatment still partially substandard and limited capacity: **In Ituri**, the Ebola Treatment Centers (ETCs) are saturated with suspected cases: Nizi, Mambasa Lolwa, Kilo, Mangala, and Mandima; 30 patients are in critical condition. **In Haut-Uélé**, the Pawa ETC lacks hygiene facilities, and the Isiro ETC lacks space, water, and a constant supply of electricity.

In Tshopo, the lack of safe blood and limited technical resources are hindering patient care. **In Bas-Uélé**, the absence of TSE/CT centers and ambulances limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 1,582 people have been vaccinated to date out of 11,703 TPL/PPL targets (14%), including 1,079 in Makiso-Kisangani, 269 in Kabondo and 234 in Mangobo; the remaining available is 2,918 doses.
- **In Ituri**, a meeting was held to discuss the composition, recruitment, and briefing of the actors in charge of vaccinating PPLs in Bunia.

1.6.2. Challenges

Insufficient doses available in Tshopo (3,000 received out of 11,000 requested), lack of mobility for contact tracing (3 vehicles required for the mobile strategy), and the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, the stockout of ERVEBO in Buta has led to a temporary suspension of vaccinations for PPL and TPL; micro-plans are lacking in the 6 priority health zones (Aketi, Ango, Bili, Buta, Ganga, and Viadana).

1.7. Continuity of Care

- **In Bas-Uélé**, 12 high-risk contacts remain under compassionate prophylactic treatment in the health zones of Buta and Ganga-Dingila; post-exposure prophylaxis of PPL continues in the Ganga health zone.
- **In Tshopo**, evaluation of CH Shukrani; monitoring of the delivery of essential generic medicines to the Lubunga health zone and monitoring of PPL; 116 of the cases received the experimental treatment, including 13 confirmed and 103 under post-exposure prophylaxis (60 treatments completed, 11 interrupted and 32 ongoing); 2 recovered have joined the response teams.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 11,962 home visits reached 43,609 people, 447 educational talks reached 12,134 people, 79 mass awareness actions reached 5,517 people and 2 advocacy campaigns reached 5 people; 2,300 students and 58 teachers were sensitized in 4 schools in Rwampara; 158 community alerts were notified.

- **In North Kivu**, 12,186 home visits reached 48,797 people, 61 educational talks reached 16,035 people, 10 mass communication actions reached 5,649 people, 4 community dialogues reached 202 people, 2 briefings reached 89 people and 2 advocacy campaigns reached 5 people; 198 community alerts were notified out of 450 expected (44%) and 9 feedback issues were resolved out of 11 (81%); a community dialogue of 43 opinion leaders was chaired by the Bapere sector chief in the Manguredjipa health zone.
- **Haut-Uélé** : 8 mass activities reached 2,395 people, 76 home visits 456 people and 60 educational talks, 433 people.
- **In Tshopo**, 2,108 people were sensitized during 7 mass activities in schools in the Mangobo health zone, and 30 people during an educational talk with local leaders from the Plateau Médical neighborhood, at the site of the CTE (Center for Treatment and Education) under construction; 3 radio programs were broadcast on barrier gestures and vaccination of people living with HIV; 32 community alerts were reported and all resolved, as well as 2 feedback sessions.
- **In Bas-Uélé**, 1,132 people were sensitized, including 708 women and 424 men, in Nzilo (Likati Health Zone), at the PK5 checkpoint (Dakwa Health Zone) and at the Duembe Primary School (Poko Health Zone); advocacy efforts were made with the Bishop of the Diocese of Buta, the Deputy Mayor, the Head of the National Education Division and the Sub-PROVED of Bondo.

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1.8.2. Challenges

Persistent community resistance: **In Tshopo**, an attack against teams was recorded in the Bethsaïda health district (Mangobo health zone) amid rumors of an "Ebola business"; 11 contacts are refusing follow-up in the Matete health district (Mangobo health zone); and a refusal to provide samples was resolved through negotiation in Kumbakisaka. **In Haut-Uélé**, a refusal to participate in the census persists in Tely (Isiro health zone); rumors are circulating that patients are being held in tents at the Isiro General Referral Hospital, and posters and leaflets have been destroyed. **In Ituri**, resistance to the Ebola Health Distribution Teams (EDTs) has been reported in Bunia, Nizi, Nia-Nia, and Aru. **In North Kivu**, a confirmed case escaped after confirmation in Masereka, and a family in Munzambaye (Butembo health zone) is refusing decontamination of their household.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 20 of the newly confirmed cases and 113 of the 173 suspected cases received SMSPS interventions; 55 test results were announced, including 20 positive results, 51 PPL supported and 72 households assisted, 23 children and 20 recovered, followed up in the community.
- **In North Kivu**, 26 people among the new confirmed cases and 165 confirmed cases under follow-up were taken care of, as well as 115 new suspects; 99 results announced, including 27 positive, 45 PPL supported, 88 households accompanied, 7 of the 14 recovered followed up; 234 people (97 men, 102 women and 35 children) benefited from psychoeducation sessions.
- **In Tshopo**, 2 confirmed cases under follow-up and 1 new suspected case benefited from SMSPS interventions; 4 results were announced, all negative; 1 caregiver and 1 household member were supported and 2 cases benefited from psychoeducation.
- **In Haut-Uélé**, 14 confirmed cases under follow-up, 1 new suspected case and 3 suspected cases under follow-up benefited from SMSPS interventions, as well as 28 accompanying persons at the CTE; the other health zones did not transmit data.

1.9.2. Challenges

In **Ituri**, there is poor coverage of SMSPS interventions around the latest confirmed cases in Ituri (Komanda and Lita) and North Kivu (Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi), where childcare facilities are lacking in 10 affected health zones. In **Haut-Uélé**, there is insufficient staff and means of transport (Boma Mangbetu and Wamba), and an absence of SMSPS providers in Tshopo (Bafwasende and Wanie-Rukula).

1.10. Logistics

1.10.1. Main actions

- In **Ituri**, 14 of the 33 organized patient transfers were successful (42%) and 26 patients were evacuated; food support was provided to 417 of the 448 hospitalized patients (93%), 14 children were receiving LNPE/SLM at the CTE (100%), 3 were in daycare, and 27 were in the community; 2 ambulances are mobilized (1 in Bambu and 1 in Nizi); 31 patients did not receive food in 4 health zones and food needs remain in Kilo (5), Lolwa (15), Mambasa (5) and Mandima (6).
- In **North Kivu**, 14 calls were received for 14 trips carried out and all were successful (100%) and 6 ambulances were mobilized out of the 11 available (55%); 40 patients (100%), 4 accompanying persons and 50 frontline providers received a hot meal, 1 confirmed unstable patient received a specific diet at the CTE in Beni; 3 separated children are being cared for at the nursery and 1 orphaned infant is on breast milk substitute.
- In **Haut-Uélé**, delivery of PPE and PCI equipment to CH Mayogo as well as mattresses and hospital beds to the CTE of Isiro; fuel management: previous balance of 1,400 litres, outflow of 300 litres, i.e. a current balance of 1,100 litres.
- In **Tshopo**, monitoring of the construction of the Meteorological Treatment Centers (CTEs) in Météo and Bafwasende, support for the deployment of vaccines and vaccinators, packaging and deployment of Infection Prevention and Control (IPC) kits to health centers, and monitoring of the deployment of Phatkin medication in Lubunga. In Bas-Uélé, support for the mobility of health zone teams for verifying alerts and contact tracing, transport of samples to the Radi One site, and chlorine supply.

1.10.2. Security

1.10.3. Main actions

- In **North Kivu**, several activities to maintain public discipline were carried out and all were 100% covered: support and security coverage of IPC teams at the Virunga dispensary (Mdrandele health district), at the Amani health center (Benengule health district) and in households in the Kanzulinzuli and Benengule health districts for decontamination; support of EDS teams for the transport of 2 community deaths from the Bundji health district to the HGR morgue and for burials in the cemeteries of Malangayi, Masiani, Bundji, Lusovovo (Katwa), Wanamahika and Mbau (Oicha), as well as the recovery of a lifeless body in Mavivi; 62 people were screened at the communal house.

1.10.4. Challenges

In **North Kivu**, there is a lack of a permanent vehicle for the security team and a lack of financial incentive for the agents.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri,

raise awareness about sexual exploitation and abuse among the teams of the answer.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- In terms of epidemiological investigations (**100% of suspected cases investigated**), performance remains consolidated and a stability of the daily proportion of contact tracing (**86.5% above the threshold** of 85.0%).
- Sustained transmission in the provinces of Ituri, North Kivu, and Haut-Uélé, with Tshopo province showing sporadic reports. However, North Kivu province presents a worrying profile well beyond its characteristic high lethality and an increasing proportion (**31.4%**) among new confirmed cases.
- As of September 8, 2026, **86 new confirmed cases**, including **19 community deaths and 24 intra-CTE deaths**, were recorded in the last 24 hours, bringing the national total to **6,843 confirmed cases and 3,310 death (case fatality rate: 48.4%)**.
- Ituri remains the epicenter of the epidemic, accounting for **79.8%** of cumulative confirmed cases and **64.0%** of new cases reported in the last 24 hours.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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